CONSULT FOR AFRICA

Grandville Medical Group: Discovery

What we would need to know before we could give you advice worth paying for

Prepared by Consult for Africa

For: the leadership and marketing team, Grandville Medical Group

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Lagos and Abuja, Nigeria

Why this exists

We came to our first conversation knowing only what is public. Three centres in Lekki, Surulere and Maitama. Aesthetic and reconstructive surgery sitting alongside trauma, intensive care, laser, imaging, laboratory, dermatology and obstetrics. A med spa sub brand. Airport pickup and an aftercare home. Thirteen years of building, and a stated intention to reach all six geopolitical zones.

That is enough to form a view and nowhere near enough to give advice. A marketing firm that recommends a campaign without knowing your margin by service line, your utilisation by site and what happens to an enquiry after it arrives is guessing, and charging you for the guess.

So this is the list. It is longer than a normal intake because you are a group rather than a clinic, and because we would rather ask everything once than come back four times.

Rough numbers and honest estimates are fine throughout. We are after the shape of the business, not audited accounts, and nobody should go away and calculate anything. Where you would rather talk something through than write it down, put "discuss" and we will cover it in the working session.

An answer of "we do not track this" is useful information rather than a gap, and in this field it is often the most useful answer available, because it points straight at what is worth starting to measure. Several of the questions below exist precisely to find those.

Sections 3, 4, 6 and 9 decide the most. If time runs short, do those four and leave the rest for the session.

Everything you send us is covered by the confidentiality undertaking in our engagement terms, and nothing identifying Grandville is published, quoted or shown to anybody outside this firm without your written clearance.

1. The group as it stands

1.1 When did each of the three centres open, and in what order?

1.2 In two sentences each, how would you describe what Lekki, Surulere and Maitama actually do today? Not what they could do, what they do.

1.3 Are they one company or several? Same shareholders throughout?

1.4 Roughly how many people work in the group, split by site, and split between clinical and non clinical?

1.5 Who sits in the group's leadership team, and which of them has a growth or commercial objective in their own targets?

1.6 Is there a board, and does it meet on a fixed rhythm?

1.7 Which of the three sites makes money, and which is carried? Please be blunt. We are not going to repeat it.

1.8 Has the group taken on external investment or development finance, and does anybody outside the family of shareholders have a say in strategy?

2. The sites, and what each one is for

2.1 For each site, what is the physical footprint in approximate terms: consulting rooms, theatres, beds, intensive care beds, imaging and laboratory?

2.2 Which services are available at all three sites, which at two, and which at only one?

2.3 Is a patient ever moved between sites, and if so, for what?

2.4 Which site is the one you would point at if you had to say "this is Grandville"?

2.5 Is Maitama run as a satellite of Lagos, or as its own business with its own leadership and its own numbers?

2.6 What is the lease or ownership position at each site, and is any of the three at risk of needing to move in the next three years?

2.7 Is a fourth site under active consideration today? If so, where, and what has been committed so far?

3. Service lines, volume and margin

3.1 List your service lines as you think of them internally, not as the website groups them.

3.2 For each, roughly what share of group revenue does it represent?

3.3 For each, is it above or below the group average on margin? An ordering is enough. We do not need percentages.

3.4 Which three service lines would you want more of, if volume simply appeared tomorrow?

3.5 Which service line would you quietly be happy to do less of?

3.6 Roughly how many aesthetic and reconstructive procedures do you perform in a typical month across the group?

3.7 Roughly how many of those are done by Dr Aranmolate personally, and how many by other surgeons?

3.8 How many trauma or emergency presentations do you see in a typical month, and what proportion are admitted?

3.9 How many deliveries and how many caesarean sections in a typical month?

3.10 What does the diagnostic and laboratory business look like, and how much of it comes from people who are not otherwise your patients?

3.11 Which service lines are growing on their own, and which have been flat for two years?

3.12 Is there a service line you have launched that did not work? What happened?

3.13 What share of group revenue is self pay, and what share is paid by a health plan, an employer or an insurer?

4. Capacity, which is the question that decides whether marketing is the answer

4.1 For each site, roughly what proportion of theatre time is used in a typical week?

4.2 For each site, roughly what proportion of consulting room sessions are used?

4.3 What is bed occupancy, and what is intensive care occupancy, in a typical month?

4.4 If a patient telephones today wanting an aesthetic consultation, how long until the first available appointment? By site, and by surgeon if it differs.

4.5 Same question for a dermatology appointment, an obstetric booking, and a scan.

4.6 Where is the genuine spare capacity in the group right now? Be specific about site, service and day of the week.

4.7 Is there any service where you are currently turning work away or referring it out because you cannot absorb it?

4.8 If demand for aesthetic surgery doubled next quarter, what breaks first? Theatre, surgeon time, beds, recovery, or the front desk?

4.9 How many surgeons and consultants work on a sessional or visiting basis rather than full time, and would they take more sessions if the work existed?

5. Where your patients come from

5.1 Do you know, even roughly, what proportion of your patients live within a short drive of each site?

5.2 For the aesthetic work specifically, how far do patients travel? What proportion come from outside Lagos, and from outside Nigeria?

5.3 Do you capture a patient's location or home address in a form you could report on?

5.4 Which Nigerian cities outside Lagos and Abuja show up most often in your patient records?

5.5 Which countries show up? Is there a pattern to them?

5.6 Does Maitama draw from Abuja only, or from the north more widely?

5.7 Do you have a sense of why a patient who could have gone to a closer provider chose you? Has anybody ever asked them systematically?

5.8 Is there a catchment you believe you should be winning and are not?

6. The enquiry, from first contact to first appointment

6.1 How can somebody reach you to ask about a procedure? List every route, including the ones you wish did not exist.

6.2 Which of those routes carries the most enquiries? Your best guess is fine.

6.3 Who answers an enquiry on each route, and is it their main job or something they fit around another one?

6.4 Within what time is an enquiry typically answered, and does that change at the weekend or after six in the evening?

6.5 Roughly how many enquiries does the group receive in a month, across everything?

6.6 Roughly what proportion of those become a booked appointment?

6.7 Roughly what proportion of booked appointments become a procedure or an admission?

6.8 If those last three numbers are not available, is there anywhere they could be reconstructed from, or would we be starting from zero?

6.9 Is an enquiry that does not convert followed up? By whom, how many times, and for how long?

6.10 Could you tell me, for last month, the name and source of every enquiry that did not convert?

6.11 Do you use a customer relationship or patient management system that holds enquiries, or do they live in a mixture of telephones, notebooks and inboxes?

6.12 Is price discussed on the first call? Who decides what is said?

6.13 What is the single most common reason an interested patient does not go ahead?

7. Referrers

7.1 Which doctors, clinics or hospitals refer patients to you, and for what?

7.2 Roughly what share of your work arrives by referral rather than directly from the patient?

7.3 Is there a list of referrers anywhere, or does it live in individual relationships?

7.4 Who, in the group, owns referrer relationships as part of their job?

7.5 Does a referring doctor receive anything back after they refer: a letter, an outcome, a call, an invitation?

7.6 Which referrers used to send you patients and have stopped? This question is usually the most valuable one on the page.

7.7 Are there referrers you would like but do not have? Name the categories if not the people.

7.8 For trauma and emergency specifically, who decides where a patient is taken, and have you ever spoken to them?

7.9 Do you receive referrals from outside Nigeria, and from whom?

8. Payers, employers and corporates

8.1 Which health plans and insurers do you have arrangements with?

8.2 Roughly what share of revenue comes through them, by site?

8.3 Are those arrangements profitable at the tariffs agreed, line by line, or are some of them carried for volume?

8.4 Do you hold any direct corporate or employer contracts, for staff health, executive screening or emergency cover?

8.5 Who owns those relationships, and when were they last reviewed?

8.6 Is there a corporate or employer segment you have decided not to pursue, and why?

8.7 What do your annual health check packages sell today, in rough volume, and who buys them?

9. The brand, and the founder's name

9.1 How do you describe Grandville internally, in one sentence, when no patient is listening?

9.2 Is there a written positioning, brand guideline or messaging document anywhere in the group? Any age, any quality.

9.3 How do you intend Grandville Medical and Laser, Grandville Medical Group and Grandville Med Spa to relate to one another? Is that a decision that has been taken or a set of names that accumulated?

9.4 Is the med spa a separate business, a sub brand, or a room inside the hospital?

9.5 Does the group's name appear on anything in the same visual language across all three sites, or has each site drifted?

9.6 What proportion of enquiries, in your estimation, arrive because of Dr Aranmolate personally rather than because of Grandville?

9.7 How does the group plan to grow its surgical capacity: more surgeons under the Grandville name, or more of Dr Aranmolate's time?

9.8 If a patient were treated by a different Grandville surgeon, would they feel they had received the thing they came for? Has that been tested?

9.9 Who speaks for the group publicly, other than the founder?

9.10 Is there any part of the group's story that you consider off limits publicly?

9.11 What do you believe patients say about you when you are not in the room, both good and bad?

9.12 Has the group had a reputational incident, a review problem or a press issue that we should know about before we write anything?

10. The inbound and travelling patient

10.1 Roughly how many patients a year travel to you from outside Nigeria?

10.2 Where do they come from, and are they Nigerian diaspora, other West Africans, or neither?

10.3 How does a travelling patient find you today?

10.4 Is there a defined pathway for them: enquiry, consultation, quotation, deposit, travel, surgery, recovery, discharge, follow up?

10.5 What is included in a price quoted to a travelling patient, and what is not?

10.6 How does the aftercare home work? Who staffs it, how long does a patient stay, and is it priced separately?

10.7 What happens if a complication appears after a patient has flown home? Is that answer written down anywhere a patient could read it before they book?

10.8 Do you have relationships with clinics or practitioners abroad who see these patients before they travel?

10.9 Has anybody ever calculated what a travelling patient is worth compared with a local one?

10.10 Is there a reason the group has not pushed this harder, beyond not having got to it?

11. Marketing as it is today

11.1 Who does marketing for the group? Names, roles, and whether it is their whole job.

11.2 Who do they report to, and who signs off what goes out?

11.3 What is the marketing budget for the year, split roughly between people, media and production?

11.4 What are you running today: social, search, paid media, print, outdoor, radio, events, influencers, public relations?

11.5 Which channels do you believe are working, and what makes you believe it?

11.6 Which have you tried and stopped?

11.7 Do you work with any agency, freelancer or production house today? What do they do and how long have they been doing it?

11.8 Who owns your website, and who can change it this afternoon without asking anybody?

11.9 Who holds the login to your social accounts, your advertising accounts and your domain?

11.10 Do clinicians contribute content, and are they willing to?

11.11 What is the rule on patient photographs and before and after images? Who takes them, who consents them, and where are they stored?

11.12 Has a campaign ever created more demand than a site could serve? What happened?

11.13 What does marketing get asked for most often by the rest of the business, and what does it never get asked for that it should?

12. Measurement

12.1 Does anybody ask a new patient how they heard about you? If so, who records it and where does it go?

12.2 Is there analytics on the website, and does anybody read it?

12.3 Can you connect a patient who booked a procedure back to the thing that made them enquire?

12.4 What marketing report, if any, goes to the leadership team, and how often?

12.5 What does the leadership team actually want to know that the report does not tell them?

12.6 What is the one number that, if it moved, would convince the group that marketing is working?

13. The plan

13.1 What has to be true for the group in twelve months for the year to have been a good one? Please answer in revenue, volume or sites rather than adjectives.

13.2 And in three years?

13.3 The six geopolitical zone ambition. Is that a funded capital plan with a sequence and a date, or a direction of travel? Both are legitimate answers and they imply very different brand decisions now.

13.4 If it is funded, which zone is next and when?

13.5 Is the growth intended to be organic, by acquisition, by partnership, or by a franchise or affiliate model?

13.6 What is the group's position on a site that carries the Grandville name but is not owned by the group?

13.7 What is the largest single risk to the plan, in your view?

13.8 What has stopped the group from doing this already?

14. How you would want to work with us

14.1 What made you take this meeting?

14.2 Have you worked with a consultancy or an agency before? What was good about it and what was not?

14.3 Who would be our day to day counterpart, and who signs the invoice?

14.4 Are you looking for a project with an end, or a function that runs?

14.5 Is there a budget envelope already in somebody's mind? If there is, saying so saves us both a round.

14.6 Is there a date or an event we should be working back from?

14.7 What would make you decide, six months in, that this had not worked?

What happens after you send this back

We read it, and within a week you get a written read of what we think the group's growth constraint actually is, which service line we would point at first, and what we would not spend money on yet. That note is yours whether or not you engage us.

Then a working session with whoever owns the numbers, and a proposal against one shape rather than six.

If something here is uncomfortable to put in writing to a firm you met once, leave it out and mark it for the session. We would rather earn it than extract it.

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